

EMERGENCY DEPARTMENT NOTE

Patient: Sarah Mitchell

DOB: June 15, 1985 (Age: 40)

MRN: CTX-2024-001

Arrival: February 15, 2026 22:15

Discharge: February 16, 2026 01:30

Provider: Dr. Michael Roberts, MD

CHIEF COMPLAINT:

Severe headache and nausea

HISTORY OF PRESENT ILLNESS:

40-year-old female presents with sudden onset severe headache starting 4 hours ago. Describes as "worst headache of life", throbbing, frontal. Associated with nausea and photophobia. No recent trauma. No fever. No vision changes. No neck stiffness. No focal neurological deficits.

TRIAGE VITALS:

BP: 165/95 mmHg | HR: 95 bpm | Temp: 98.6 F

RR: 18/min | SpO2: 99% RA | Pain: 9/10

PHYSICAL EXAMINATION:

General: Moderate distress due to pain

HEENT: PERRLA. No papilledema. TMs clear.

Neck: Supple. No meningismus. No lymphadenopathy.

Neurological: Alert and oriented x3. CN II-XII intact.

Motor 5/5 throughout. Sensation intact. DTRs 2+ symmetric.

Negative Kernig and Brudzinski signs.

DIAGNOSTIC STUDIES:

CT Head without contrast: No acute intracranial abnormality.

No hemorrhage, mass, or midline shift.

TREATMENT:

- IV access established
- Ondansetron 4mg IV for nausea
- Ketorolac 30mg IV for pain
- Normal saline 1L IV bolus
- Metoclopramide 10mg IV

REASSESSMENT:

Pain improved to 3/10. Nausea resolved. BP 138/82.

Patient tolerating oral fluids.

DIAGNOSIS:

Severe migraine headache

DISPOSITION:

Discharged home in stable condition. Prescriptions provided. Follow-up with PCP in 1 week. Return precautions discussed.